

Is HRT right for you?

A short, evidence-based guide to help you decide what to ask — and to walk into your appointment clear, not uncertain. Grounded in current NICE guidance.

You don't have to decide alone, and you don't have to know the answer before you go in. This is simply what to understand, and what to bring.

THE BASICS

What HRT actually does

Hormone replacement therapy tops up the oestrogen — and, if you still have a womb, progesterone — that falls during the menopause transition. It is the **most effective treatment** for hot flushes and night sweats, and NICE recommends it as the first-line option for them. It also helps disrupted sleep, mood changes arising in perimenopause, and vaginal dryness, and it protects bone strength over the longer term.

THE FEAR, ADDRESSED

“Isn't it dangerous?”

Most of that worry traces to early 2002 reports that were widely misread. The picture has been re-examined since. For most women under 60, or within ten years of menopause, the benefits for troublesome symptoms outweigh the risks — and NICE states HRT is **unlikely to affect life expectancy** either way.

THE NUANCE

Type matters

Risk depends heavily on the **type** of HRT. Oestrogen through the skin — a patch or gel — carries a lower clot risk than tablets, which is why it is often preferred for women with certain histories. This is exactly the kind of detail worth asking about.

IMPORTANT

“Things to discuss first” are not “you can't”

A history of breast cancer, blood clots, liver disease or migraine with aura does not automatically rule HRT out. What they do is shift the conversation — towards a specialist, a particular type, or non-hormonal options if HRT truly isn't suitable.

One flag is different: unexplained vaginal bleeding should always be checked by a doctor before starting HRT, and promptly, whatever you decide.

WHAT HRT IS RECOGNISED TO HELP

- **Hot flushes & night sweats** — the symptoms it helps most; first-line per NICE.
- **Disrupted sleep** — often improves as night sweats settle.
- **Mood, anxiety & irritability** arising in perimenopause.
- **Vaginal dryness & discomfort** — sometimes treatable with local vaginal oestrogen alone, without whole-body HRT.
- **Bone strength** — a longer-term protective benefit.

Your conversation plan

Clinicians decide with you, not for you. Bring clear answers and clear questions, and the appointment works far better.

FIRST, NOTE WHERE YOU ARE

Your symptom snapshot

Rate each from 0 (none) to 3 (severe) over the last few weeks. One clear picture is worth more than trying to remember on the day.

SYMPTOM	NONE 0	MILD 1	MOD 2	SEVERE 3
Hot flushes & night sweats	☐	☐	☐	☐
Disrupted sleep	☐	☐	☐	☐
Mood, anxiety, irritability	☐	☐	☐	☐
Vaginal dryness / discomfort	☐	☐	☐	☐
Joint aches, brain fog, fatigue	☐	☐	☐	☐

THEN, ASK THESE

Questions worth asking your GP

- 1 Given my symptoms, is HRT a reasonable option for me — and what are the alternatives?
- 2 Which **type** would suit me best — patch, gel or tablet — and why?
- 3 If I have any history you should know about, does that change the type you'd suggest, rather than rule HRT out?
- 4 If my main issue is dryness, could **local vaginal oestrogen** help on its own?
- 5 What would a typical **review** look like — when, and what would we check?

GOOD TO KNOW

HRT is usually **reviewed at three months**, then yearly. If you've had a hysterectomy, **oestrogen-only** HRT is usually appropriate, which simplifies the choice.

BEFORE YOU GO

Jot down **when** your symptoms started and **how** they affect your day. Specifics — “waking three times a night for four months” — help more than “I'm tired”.

ALWAYS WORTH CHECKING PROMPTLY

If you have **any unexplained vaginal bleeding** — between periods, after sex, or after menopause — mention it to your doctor and ask to have it looked into, regardless of anything else. This should be checked before starting HRT.

Turn this into your personalised guide

Answer a few questions at **The Hormone Blueprint** and get a conversation guide tailored to your own symptoms and history.